

Dementia - Self-Care

CCG: C-0073 (CCG)

[Link to Codes](#)

MCG Health
Chronic Care
28th Edition

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Assessment

Initial Consideration

Q1. Case Manager: Are there any barriers preventing patient participation in the program? (Check all that apply.) **QM**

- Behavioral or cognitive problems (eg, inappropriate behaviors or very poor memory) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Advance Care Planning [CCG](#)
- Hearing problems Hearing [CCG](#)
- Language barrier Language [CCG](#)
- Vision problems Vision [CCG](#)
- No
- Unknown [P1]

Q2. Patient: What would you like to talk about today? (Check all that apply.) **QM**

- Emergency or urgent problems (eg, social determinants of health, intimate partner violence, suicidal ideation)[A] Social Determinants of Health Screening Assessment [CCG](#) Intimate Partner Violence [CCG](#) Suicide or Personal Harm Risk [CCG](#)
- Condition information
- Provider coordination (eg, next appointment, treatment clarification) Appointment Scheduling [CCG](#)
- Medications
- Complications
- Monitoring and management
- Other health problems (eg, comorbid conditions, new symptoms)
- Other, specify: _____
- Nothing
- Unknown

Q3. Patient: Do you have any changes since the last time we talked? (Check all that apply.) **QM**

- Adhering to treatment or medication problems Readiness to Change [CCG](#) Medication Adherence [CCG](#)
- Administering medication Home Infusion Management [CCG](#) Injectable Medications [CCG](#)
- Affording medicine or supplies Financial Status and Benefits [CCG](#)
- Aspiration Aspiration Risk Management [CCG](#)
- Behavioral health provider appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Blood pressure care Blood Pressure Monitoring [CCG](#)

- Blood sugar care Blood Glucose Monitoring [CCG](#) HbA1c Monitoring [CCG](#)
- Blood thinner care Anticoagulation Management [CCG](#)
- Bowel care Bowel Management [CCG](#)
- Bowel changes Bowel Symptoms [CCG](#)
- Caregiver Caregiver Resources Evaluation [CCG](#)
- Chemotherapy Chemotherapy [CCG](#)
- Cognitive impairment Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Communication or speech problems Speech and Communication Problems [CCG](#)
- Community resource Community Resources [CCG](#)
- Dehydration care Dehydration Monitoring and Management [CCG](#)
- Depression symptoms Depression Screening, Adult [CCG](#)
- Dialysis care Dialysis Management [CCG](#)
- Doctor or specialist outpatient appointment, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Energy level changes Energy Level Changes [CCG](#)
- Enteral nutrition care Enteral Nutrition Management [CCG](#)
- Exercising and staying active Exercise [CCG](#)
- Fatigue or weakness Fatigue or Weakness [CCG](#)
- Fluid restriction Fluid Restriction [CCG](#)
- Functional status changes Activities of Daily Living [CCG](#) Instrumental Activities of Daily Living [CCG](#)
- Home infusion Home Infusion Management [CCG](#)
- Hospital or emergency room visit, specify: _____ Hospitalization Risk Assessment [CCG](#) Post-Hospitalization Follow-Up [CCG](#)
- Medication changes Medication Changes [CCG](#)
- Nebulizer care Nebulizer Management [CCG](#)
- Neurologic function changes Neurologic Function Changes [CCG](#)
- Ostomy care Ostomy Management [CCG](#)
- Oxygen care Oxygen Management [CCG](#)
- Pain, chronic Chronic Pain [CCG](#)
- Pain, new or general Pain Assessment [CCG](#)
- Parenteral nutrition care Parenteral Nutrition Management [CCG](#)
- Peak flow meter care Peak Flow Meter Use [CCG](#)
- Peripheral vascular (eg, lower leg redness or swelling) Peripheral Vascular Problems [CCG](#)
- Physical impairments Physical Impairments [CCG](#)
- Planning for long-term care needs Advance Care Planning [CCG](#)
- Procedure, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Pulmonary treatments Pulmonary Treatments [CCG](#)
- Rehabilitation therapy, specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Respiratory changes Respiratory Symptoms [CCG](#)
- Safety Safety [CCG](#)
- Scheduling care Appointment Scheduling [CCG](#)
- Sleeping problems Sleep [CCG](#)
- Social support Social Support [CCG](#)
- Stasis ulcer care Stasis Ulcer Management [CCG](#)
- Testing (eg, laboratory, imaging), specify: _____ Follow-Up - Provider Appointment [CCG](#)
- Tracheostomy care Tracheostomy Management [CCG](#)
- Transportation Transportation [CCG](#)

- Understanding healthcare needs Health Literacy [CCG](#)
- Urinary catheter care Urinary Catheter Management [CCG](#)
- Urinary changes Urinary Symptoms [CCG](#)
- Venous thromboembolism symptoms Venous Thromboembolism [CCG](#)
- Ventilator care Ventilator Management [CCG](#)
- Weight gain Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Weight loss Nutritional Status, Adult [CCG](#) Nutritional Status, Pediatric [CCG](#) Nutritional Status, Infant [CCG](#)
- Wound care Wound Management [CCG](#)
- Other, specify: _____
- No
- Unknown Hospitalization Risk Assessment [CCG](#)

Condition Knowledge

Q1. Patient: What causes dementia? (Check items understood.)

- Alcoholism
- Alzheimer disease
- Brain tumors
- Depression
- Family history (genetics)
- Head injuries
- Hydrocephalus
- Infection: neurosyphilis, HIV
- Medicine: anticholinergics, barbiturates, bromides, glucocorticoids, haloperidol, opiates
- Metals: lead, mercury
- Neurologic conditions (eg, Parkinson disease, Lewy body dementia)
- Vitamin deficiency: B12, thiamine
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q2. Case Manager: Rate the patient's understanding of the causes of dementia. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q3. Patient: What are the symptoms of dementia? (Check items understood.)

- Believing others are out to harm or steal from you (paranoia)
- Confusion
- Difficulty planning and carrying out tasks like driving, balancing checkbook, or following a recipe
- Disorientation to day, date, time, and location
- Easily distracted
- Getting lost in familiar or unfamiliar places
- Mood changes: aggression, agitation, depression
- No interest in pleasurable activities
- Poor hygiene
- Poor judgment
- Poor reasoning
- Problems remembering words to say
- Problems with walking and balance
- Repeating the same information over again
- Seeing or hearing things that others do not see or hear
- Short-term memory loss
- Taking actions without thinking first
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]

- Not all appropriate information known [P1]

Q4. Case Manager: Rate the patient's understanding of the symptoms of dementia. **QM**

- 90% or more
- 75% to 89% [P1]
- 50% to 74% [P1]
- Less than 50% [P1]
- Unknown [P1]

Q5. Patient: How can you care for dementia to keep it from getting worse and causing problems? (Check items understood.) **QM**

- Adhere to self-care plan
- Break complex projects into smaller tasks
- Exercise
- Follow a daily care plan
- Follow an eating (diet) plan
- Keep all doctor appointments
- Keep a vaccination schedule
- Know when to contact the doctor
- Maintain social interaction and activity
- Manage other diseases
- Manage situation or event triggers
- Manage stress
- Safety awareness (fall prevention)
- Seek emergency care for signs of heart or blood vessel problems or stroke
- Set realistic expectations for feeling better, managing symptoms, and recovery
- Set small goals every day
- Stop using alcohol, tobacco, or drugs
- Take medicines as ordered
- Other, specify: _____
- Unknown [P1]
- Nothing [P1]
- Not all appropriate information known [P1]

Q6. Case Manager: Rate the patient's understanding of how to care for dementia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What activities would you be able to do if you didn't have problems with your dementia? (Check all that apply.) **QM**

- Activities of daily living (ADL) without assistance [P2]
- Cook [P2]
- Decision making (eg, medical care) [P2] Advance Care Planning [CCG](#)
- Drive [P2]
- Exercise [P2]
- Go out with friends [P2]
- Go to events [P2]
- Play sports [P2]
- Play with children or grandchildren [P2]
- Shop [P2]
- Travel [P2]
- Other, specify: _____ [P2]
- Unknown [P1]

Provider Information

Q1. Patient: What doctor manages your dementia? **QM**

- Psychiatrist or psychologist, name and phone number: _____ [Q2]
- Neurologist, name and phone number: _____ [Q2]
- Primary doctor, name and phone number: _____
- Other specialist, name and phone number: _____ [Q2]

- Don't have doctor [P1]
- Unknown [P1]

Q2. Patient: When was your last visit to your doctor who manages your dementia? **QM**

- Less than 1 month ago
- 1 to 3 months ago [P2]
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q3. Patient: When was your last visit with your primary care doctor? **QM**

- Within the past 3 months
- 4 to 6 months ago [P2]
- More than 6 months ago [P2]
- Unknown [P2]
- Never [P2]

Q4. Patient: What other specialists have you seen within the last 12 months? (Check all that apply.) **QM**

- Behavioral health provider
- Cardiologist
- Dentist
- Dietitian
- Endocrinologist
- Gastroenterologist
- General surgeon
- Genetic counselor
- Geriatrician
- Gynecologist
- Hematologist
- Hepatologist
- HIV/AIDS
- Infectious disease
- Nephrologist
- Neurologist
- Neurosurgeon
- Obstetrician
- Oncologist
- Ophthalmologist
- Orthopedic surgeon
- Orthopedist
- Otolaryngologist
- Pain management
- Psychiatrist
- Plastic surgeon
- Podiatrist
- Pulmonologist
- Rheumatologist
- Transplant specialist
- Urologist
- Other, specify: _____
- None [P1]
- Unknown [P1]

Q5. Patient: How many dementia-related emergency department visits or hospital admissions have you had in the past 12 months?
QM

- 2 or more [P2] Hospitalization Risk Assessment [CCG](#)
- 1 [P2]
- None
- Other, specify: _____
- Unknown [P2]

Q6. Patient: Do you have a written self-care plan from your doctor on how to care for dementia?[B] **QM**

- Yes [Q7]
- No [P1] [P2]
- Unknown [P1] [P2]

Q7. Patient: What are the components of your dementia self-care plan? (Check all that apply.) **QM**

- Crisis management plan
- Exercise plan
- Knowing what medicines you take and why and how to take them
- Managing side effects of medicines
- Nutrition plan
- Recognizing the symptoms of gradual worsening or problems
- Regular follow-up appointments for wellness checks
- When to call the doctor and when to get emergency help
- Other, specify: _____ [P1]
- Unknown [P2]
- Not applicable
- Not all appropriate information known [P2]

Q8. Case Manager: Is the patient able to understand and adhere to the self-care plan? **QM**

- Yes, independently
- Yes, with occasional reminders or caregiver support
- Yes, with frequent reminders or caregiver support
- No [P3]
- Unknown [P3]

Medication Information

Q1. Patient: Has any medicine for dementia been prescribed by your doctor? If so, is it taken as ordered? **QM**

- Yes, and taken as ordered [Q2]
- Yes, but not taken as ordered [Q2] [P4] Medication Adherence [CCG](#)
- No medicine prescribed [P4]
- Unknown [P4]

Q2. Patient: What medicines do you take for dementia? (Check all that apply.) **QM**

- Antidepressants (eg, citalopram, sertraline) [Q3] [P5] [P6]
- Cholinesterase inhibitors (eg, donepezil, galantamine, rivastigmine) [Q3] [P5] [P7]
- Memantine [Q3] [P5] [P8]
- Typical antipsychotics (eg, chlorpromazine, haloperidol, thioridazine) [Q3] [P5] [P9]
- Other, specify: _____ [Q3] [P4] [P5]
- Not applicable [P4]
- Unknown [P5]

Q3. Patient: Are you having side effects from the dementia medicine? (Check all that apply.)

- Confusion [P5]
- Dizziness [P5]
- Dry mouth [P5]
- Gait disturbance [P5]
- Headache [P5]
- Nausea [P5]
- Sedation [P5]
- Sleep problems [P5]
- Other, specify: _____ [P5]
- None
- Unknown [P5]
- Not applicable

Q4. Case Manager: Rate the patient's understanding of the purpose and side effects of the medications for dementia. **QM**

- 90% or more
- 75% to 89% [P1] [P5]
- 50% to 74% [P1] [P5]
- Less than 50% [P1] [P5]

- Unknown [P1] [P5]

Complications

Q1. Patient: How does your ability to do activities compare with that of 6 months ago? **QM**

- Stable
- Improved
- Declined [P10] [P11] Activities of Daily Living [CCG](#)
- Unknown [P11]

Q2. Patient: Are you having any problems managing care due to physical impairments? (Check all that apply.) **QM**

- Doing activities of daily living (eg, bathing, dressing, or eating) [P10] [P11] Activities of Daily Living [CCG](#)
- Doing instrumental activities of daily living (eg, shopping, cooking, or housework) [P10] [P11] Instrumental Activities of Daily Living [CCG](#)
- Equipment problems [P10] [P11] Assistive Devices [CCG](#)
- Getting caregiver help [P10] [P11] Caregiver Resources Evaluation [CCG](#)
- Getting transportation [P11] Transportation [CCG](#)
- Other, specify: _____ [P10] [P11]
- No problems
- Unknown [P10] [P11]

Q3. Patient: Have there been any falls? **QM**

- Yes [P10]
- No
- Unknown [P10]

Q4. Patient: Have you had rehabilitation in the last 6 months? (Check all that apply.) **QM**

- Cognitive therapy
- Occupational therapy
- Physical therapy
- Speech therapy (including for swallowing evaluation and treatment of swallowing disorder)
- Other, specify: _____
- All appropriate therapy received
- None [P11]
- Unknown [P11]

Q5. Patient: How do you get around? (Check all that apply.) **QM**

- Walk without help (independent without device use) [P10] Safety [CCG](#)
- Walk with help (eg, use of cane or brace) [P10] [P11] Safety [CCG](#)
- Walk at home but use a wheelchair when going out [Q6] [P10] [P11] Safety [CCG](#)
- Walk with someone's help [P10] [P11] Safety [CCG](#)
- Walk but tire easily or are unsteady [P10] [P11] Safety [CCG](#)
- Wheelchair-bound [Q6] [P10] [P11] Safety [CCG](#)
- Bed-bound [Q7] [P10] [P11] Safety [CCG](#)
- Other, specify: _____ [P11] Safety [CCG](#)
- Unknown [P11] Safety [CCG](#)

Q6. Patient: Are there any problems managing wheelchair tasks? (Check all that apply.) **QM**

- Crossing streets [P11]
- Equipment problems [P10] [P11]
- Folding and opening wheelchair (manual only) [P10] [P11]
- Maneuvering through tight spaces such as hallways, over uneven or irregular terrain, over curbs, and up ramps [P10] [P11]
- Opening, closing, and going through doors [P11]
- Operating control system for power wheelchair [P10] [P11]
- Performing wheelies (manual only) [P10] [P11]
- Reaching for objects above and below [P10] [P11]
- Transfers (eg, chair, bed) [P10] [P11]
- Using basic skills such as braking, using armrests and footrests, and self-propulsion [P10] [P11]

- Other, specify: _____ [P10] [P11]
- No problems
- Unknown [P10] [P11]
- Not applicable

Q7. Patient: Are there any problems managing tasks while bed-bound? (Check all that apply.) **QM**

- Moving in bed [P11]
- Picking up objects [P10] [P11]
- Reaching for objects above and below [P10] [P11]
- Other, specify: _____ [P10] [P11]
- No problems
- Unknown [P10] [P11]
- Not applicable

Q8. Patient: Do you get regular exercise? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q9. Patient: Do you have any swallowing problems? (Check all that apply.)

- Chewing of food or total eating time takes longer than it used to. [Q10] [P12]
- Choke or cough when swallowing [Q10] [P12]
- Drool when eating or drinking [Q10] [P12]
- Food feels like it is stuck in throat. [Q10] [P12]
- Frequent lung infections (eg, pneumonia) [Q10] [P12]
- It is getting harder to swallow. [Q10] [P12]
- Pain with swallowing [Q10] [P12]
- Problems paying attention while eating [Q10] [P12]
- Slurring of speech [Q10] [P12]
- Spitting food out of mouth or regurgitation [Q10] [P12]
- Tongue thrusting or poor tongue control [Q10] [P12]
- Trouble swallowing [Q10] [P12]
- None
- Unknown [P12]

Q10. Patient: Are these problems new or have they recently gotten worse?

- Yes, new [P12]
- Yes, gotten worse [P12]
- No
- Unknown [P12]

Q11. Patient: How often do you require supervision while eating?

- Every meal [P12]
- Most meals during day [P12]
- At least 1 meal per day [P12]
- Occasionally [P12]
- Never
- Unknown [P12]

Q12. Patient: How often does food or fluid go down the wrong pipe?

- Once to several times per week [P12]
- Once to several times per month [P12]
- Every few months [P12]
- Never had aspiration episode
- Unknown [P12]

Q13. Patient: Have you been evaluated by a speech or swallowing therapist in the last 3 months?

- Yes
- No [P12]
- Unknown

Q14. Patient: When was your last test to see how well you can swallow?

- Less than 6 months ago
- 6 to 12 months ago
- More than 12 months ago
- Never had swallowing test
- Unknown

Q15. Patient: What best describes your current eating plan? (Check all that apply.)

- Nothing by mouth; you use other method of feeding (eg, enteral or parenteral nutrition). [Q16] Enteral Nutrition Management [CCG](#)
Parenteral Nutrition Management [CCG](#)
- Someone helps you eat. [Q16]
- Someone is always with you while you eat, but help is not usually needed. [Q16]
- You are generally safe by yourself if you follow your eating plan. [Q16]
- Your eating plan has a few limits with small changes in eating style.
- No trouble with normal eating plan
- Unknown [P12]

Q16. Patient: Do you follow your eating plan?

- Yes
- No [P12]
- Unknown [P12]

Q17. Patient: Has your mood changed over the past 2 weeks?(1)(2) [PHQ-2 Calculator](#) **QM**

- Yes [Q18]
- No
- Unknown

Q18. Patient: What changes in your mood have you had? (Check all that apply.) **QM**

- Change in usual activity (eg, working excessively, no longer talking to friends, or going on shopping sprees) Depression Screening, Adult [CCG](#)
- Down, hopeless, or depressed Depression Screening, Adult [CCG](#)
- Forgetting a lot or getting angry or frustrated Depression Screening, Adult [CCG](#)
- Little interest or pleasure in doing things Depression Screening, Adult [CCG](#)
- Quick mood changes Depression Screening, Adult [CCG](#)
- Other, specify: _____ Depression Screening, Adult [CCG](#)
- Not applicable

Q19. Patient: Are there any current sleeping problems? (Check all that apply.) **QM**

- Falling asleep [P13]
- Staying asleep [P13]
- Waking up too early [P13]
- Not feeling rested after sleeping [P13]
- Other, specify: _____ [P13]
- None
- Unknown [P13]

Q20. Patient: What do you do to help sleep? (Check all that apply.)

- Decrease caffeine
- Don't eat big meals before bedtime
- Get daily exercise
- Go to bed and get up at routine times of the day
- Keep journal of thoughts
- Sleep in bed, not on chair or sofa
- Sleep in dark, quiet room
- Sleep with head elevated, not lying flat
- Stopped drinking
- Stopped smoking
- Take a warm bath or shower before bedtime
- Take medicines, specify: _____ [Q21]
- Took distractions out of bedroom (eg, TV)

- Use equipment (eg, oxygen or CPAP)
- Other, specify: _____
- Nothing
- Unknown [P13]
- Not all appropriate sleep promoting activities being used [P13]

Q21. Patient: Do the medicines work to help sleep?

- Yes
- No [P13]
- Unknown
- Not applicable

Q22. Case Manager: Rate the patient's understanding of the potential complications of dementia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Monitoring and Management

Q1. Patient: Do you sometimes have problems thinking or remembering? If so, what problems do you have? (Check all that apply.)

QM

- Activities of daily living tasks (eg, bathing, oral hygiene) [Q2] [P14]
- Appointment forgetfulness [Q2] [P14]
- Concentrating during conversations [Q2] [P14]
- Date (ie, time, month, year) [Q2] [P14]
- Decisiveness [Q2] [P14]
- Impulsive [Q2] [P14]
- Instrumental activities of daily living tasks (eg, grocery shopping, paying bills) [Q2] [P14]
- Location (ie, getting lost) [Q2] [P14]
- Medicine forgetfulness [Q2] [P14]
- Names or people [Q2] [P14]
- Past events, recent [Q2] [P14]
- Problem solving [Q2] [P14]
- Safety steps (eg, locking doors, turning off stove) [Q2] [P14]
- Train of thought [Q2] [P14]
- No
- Unknown [Q2] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)

Q2. Patient: How does your current status of thinking or remembering compare with that of 6 months ago?

- About the same
- Better
- Worse [B1] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Preliminary Depression Screening [CCG](#)
- Unknown [B1] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Preliminary Depression Screening [CCG](#)

Q3. Patient: Do you have problems with day-to-day life because of trouble thinking or remembering? **QM**

- Yes [Q4] [B1] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Preliminary Depression Screening [CCG](#)
- No
- Unknown [Q4] [B1] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Preliminary Depression Screening [CCG](#)

Q4. Patient: Is there someone who helps you with your self-care and daily responsibilities? If so, who? **QM**

- Yes, specify: _____ Caregiver Resources Evaluation [CCG](#)
- No [B1] [P14]
- Unknown [B1] [P14] Behavioral Health and Cognition Evaluation with Caregiver [CCG](#) Preliminary Depression Screening [CCG](#)

Q5. Patient: How much do you weigh and how tall are you? **QM**

- Weight: _____
- Height: _____
- Unknown [P15]

Q6. Case Manager: What is the patient's body mass index (BMI)? BMI Calculator

- Less than 18.5 [P15] [P16] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG
- 18.5 to 24.9
- 25 to 29.9 [P15] [P16] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- 30 or greater [P15] [P16] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- Unknown [P15]

Q7. Patient: Do you know what your body weight should be?

- Yes, it should be: _____
- No [P15]
- Unknown [P15]

Q8. Patient: What do you eat and drink in a typical day? (Check all reported.)

- Alcohol, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Dairy foods, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Fats, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Fruits, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Fruit juices, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Grains, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Meats (beef, chicken, fish), specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Milk, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Sodas, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Supplements, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Sweets, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Vegetables, specify types and number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Water, specify number of daily servings: _____ [Q9] [Q10] [Q11] [Q12] [Q13]
- Other, specify: _____ [Q9] [Q10] [Q11] [Q12] [Q13] [P15]
- No food by mouth [Q13] [P15] [P16]
- Unknown [Q9] [Q10] [Q11] [Q12] [Q13] [P15] [P16]

Q9. Patient: How often do you eat each day? (Check all reported.) **QM**

- 3 meals or more [P15] [P16]
- 2 meals
- 1 meal [P15] [P16]
- 0 meals [P15] [P16]
- 2 snacks or more [P15] [P16]
- 1 snack
- 0 snacks
- Other, specify: _____ [P15] [P16]
- Not applicable
- Unknown [P15] [P16]

Q10. Patient: How much fluid (such as water, juice, coffee, tea, milk) do you have per day? **QM**

- Less than 3 cups (720 mL) [P15] [P16]
- 3 to 5 cups (720 mL to 1.2 L) [P15] [P16]
- More than 5 cups (1.2 L)
- Unknown [P15] [P16]

Q11. Patient: Are you trying to gain or lose weight?

- Trying to gain weight [P15] Nutritional Management to Gain Weight  CCG Body Weight Planner  CCG
- Trying to lose weight [P15] Nutritional Management to Lose Weight  CCG Body Weight Planner  CCG
- Neither
- Unknown [P15]

Q12. Patient: Does your doctor want you to follow a special diet program? (Check all that apply.) **QM**

- Yes, DASH (low sodium) [P15] Diet Knowledge Insufficient - DASH Diet  CCG

- Yes, heart healthy [P15] Diet Knowledge Insufficient - Heart Healthy [CCG](#)
- Yes, kidney diet [P15] Diet Knowledge Insufficient - Renal Dysfunction [CCG](#)
- Yes, low carb or low glycemic index [P15] Diet Knowledge Insufficient - Low Glycemic Index [CCG](#)
- Yes, pregnancy diet [P15] Diet Knowledge Insufficient - Pregnancy [CCG](#)
- Yes, other: _____ [P15]
- No [P15]
- Unknown [P15]

Q13. Patient: Do you get nutritional treatments or have an altered diet? If yes, why? (Check all that apply.) **QM**

- Behavioral health provider seen, specify: _____ [P16]
- Dental soft diet, specify: _____ [P16]
- Diet supplements, specify: _____ [P16]
- Dietitian seen or following, specify: _____ [P16]
- Enteral nutrition (tube feeding), specify: _____ [P16] Enteral Nutrition Management [CCG](#)
- IV fluids (hydration), specify: _____ [P16]
- Liquid diet, specify: _____ [P16]
- Mechanical soft diet, specify: _____ [P16]
- Parenteral nutrition, specify: _____ [P16] Parenteral Nutrition Management [CCG](#)
- Procedure(s) done by a doctor in office, specify: _____ [P16]
- Pureed diet, specify: _____ [P16]
- Speech-language pathology (SLP), specify: _____ [P16]
- Supervised during nutritional intake, specify: _____ [P16]
- Surgery, specify: _____ [P16]
- Thickened liquid diet, specify: _____ [P16]
- Other, specify: _____ [P16]
- No
- Unknown [P15]

Q14. Patient: Do you know how to read food labels?

- Yes
- No [P17]
- Unknown [P17]

Q15. Patient: Do you follow a meal plan to help with your dementia? **QM**

- Yes
- No [P2]
- Unknown [P2]

Q16. Patient: Are you having any problems with your dementia self-care needs? (Check all that apply.) **QM**

- Affording medicine or supplies [P2] [P3] Financial Status and Benefits [CCG](#)
- Behavior indicating cognitive impairment [P2] [P3] Cognitive Impairment [CCG](#) Behavioral Health and Cognition Evaluation with Caregiver [CCG](#)
- Blood pressure care [P2] [P3] Blood Pressure Monitoring [CCG](#)
- Blood sugar care [P2] [P3] Blood Glucose Monitoring [CCG](#)
- Blood thinner care [P2] [P3] Anticoagulation Management [CCG](#)
- Bowel program [P2] [P3] Bowel Management [CCG](#)
- Caregiver help needed [P2] [P3] Caregiver Resources Evaluation [CCG](#)
- Community resource information needed [P2] [P3] Community Resources [CCG](#)
- Exercising and staying active [P2] [P3] Exercise [CCG](#)
- Fatigue or weakness [P2] [P3] Fatigue or Weakness [CCG](#)
- Feeding tube (enteral nutrition) use and care [P2] [P3] Enteral Nutrition Management [CCG](#)
- Oxygen use and care [P2] [P3] Oxygen Management [CCG](#)
- Planning for long-term care needs [P2] [P3] Advance Care Planning [CCG](#) Palliative or End-of-Life Evaluation [CCG](#)
- Safety in the home [P2] [P3] Safety [CCG](#)

- Skin care management [P2] [P3] Pressure Injury Monitoring and Management [CCG](#)
- Sleeping problems [P2] [P3] Sleep [CCG](#)
- Social support [P2] [P3] Social Support [CCG](#)
- Taking medicine as prescribed [P2] [P3] Medication Adherence [CCG](#)
- Transportation for care [P2] [P3] Transportation [CCG](#)
- Trouble gaining weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Gain Weight [CCG](#)
- Trouble losing weight [P2] [P3] Nutritional Status, Adult [CCG](#) Nutritional Management to Lose Weight [CCG](#)
- Trouble making lifestyle changes [P2] [P3] Readiness to Change [CCG](#)
- Trouble understanding how to do self-care [P2] [P3] Health Literacy [CCG](#)
- Urinary catheter care [P2] [P3] Urinary Catheter Management [CCG](#)
- Wound management [P2] [P3] Wound Management [CCG](#)
- Wound care [P2] [P3] Pressure Injury Monitoring and Management [CCG](#)
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P2] [P3]

Q17. Case Manager: Rate the patient's understanding of the monitoring and management of dementia. **QM**

- 90% or more
- 75% to 89% [P1] [P2]
- 50% to 74% [P1] [P2]
- Less than 50% [P1] [P2]
- Unknown [P1] [P2]

Other Health Considerations

Q1. Patient: Do you currently smoke or use tobacco products? (Check all that apply.) **QM**

- Yes, I smoke. [Q2] [Q4] [Q5] [Q6]
- Yes, I use other tobacco product; specify: _____ [Q3] [Q4] [Q5] [Q6]
- No, I quit. [Q4] [Q7]
- No, I have never smoked or used other tobacco products.
- Unknown

Q2. Patient: How much do you smoke?

- 2 packs a day or more [P18]
- More than 1 but less than 2 packs a day [P18]
- 1 pack a day [P18]
- Less than 1 pack a day [P18]
- Unknown
- Not applicable

Q3. Patient: How often do you use other tobacco products?

- 3 or more times a day [P18]
- 1 or 2 times a day [P18]
- A few times a week [P18]
- A few times a month [P18]
- A few times a year [P18]
- Unknown [Q6]
- Not applicable

Q4. Patient: How old were you when you started smoking or using tobacco products?

- Specify age: _____
- Unknown

Q5. Patient: What triggers you to want to smoke or use tobacco? (Check all that apply.)

- After meals [P18]
- Alcohol use [P18]
- Being around people who are smoking [P18]
- Driving or talking on the phone [P18]
- Stress [P18]

- Other, specify: _____ [P18]
- Nothing [P18]
- Unknown [P18]
- Not applicable

Q6. Patient: Are you ready to stop smoking or using tobacco products? **QM**

- Yes [P18]
- No [P18] Readiness to Change [CCG](#)
- Unknown [P18] Readiness to Change [CCG](#)

Q7. Patient: When did you quit smoking or using tobacco products?

- More than 1 year ago [P19]
- Within the last year [P19]
- Unknown

Q8. Patient: How often did you have a drink with alcohol in the past year? [C] **QM**

- Never
- Monthly or less often [Q9]
- 2 to 4 times per month [Q9]
- 2 to 3 times per week [Q9]
- 4 or more times per week [Q9]
- Unknown

Q9. Patient: How many drinks did you have on a typical day in the past year? **QM**

- None
- 1 to 2
- 3 to 4 [P20] Alcohol Misuse [CCG](#)
- 5 to 6 [P20] Alcohol Misuse [CCG](#)
- 7 to 9 [P20] Alcohol Misuse [CCG](#)
- 10 or more [P20] Alcohol Misuse [CCG](#)
- Unknown

Q10. Patient: How often did you have 6 or more drinks on one occasion in the past year? **QM**

- Never
- Less than monthly
- Monthly [P20] Alcohol Misuse [CCG](#)
- Weekly [P20] Alcohol Misuse [CCG](#)
- Daily or almost daily [P20] Alcohol Misuse [CCG](#)

Q11. Patient: Do you have any risk factors for alcohol misuse? (Check all that apply.) **QM**

- Comorbid behavioral health diagnosis (eg, depression, posttraumatic stress disorder) Alcohol Misuse [CCG](#)
- Family history Alcohol Misuse [CCG](#)
- Life-changing event (eg, death of a loved one, chronic condition diagnosis) Alcohol Misuse [CCG](#)
- Traumatic experience in childhood Alcohol Misuse [CCG](#)
- Other, specify: _____ Alcohol Misuse [CCG](#)
- None
- Unknown

Q12. Patient: What vaccinations have you had? (Check all that apply.) **QM**

- Coronavirus disease 2019 (COVID-19) [Q13]
- *Haemophilus influenzae* type b (Hib) [Q13] [Q14]
- Hepatitis A [Q13]
- Hepatitis B [Q13]
- Human papillomavirus (HPV) [Q13]
- Influenza [Q13] [Q14]
- Measles, mumps, rubella (MMR) [Q13]
- Meningococcal 4-valent conjugate (MenACWY) or polysaccharide (MPSV4) [Q13]
- Meningococcal B (MenB) [Q13]

- Pneumococcal 13-valent conjugate (PCV13) [Q13] [Q15]
- Pneumococcal 15-valent conjugate (PCV15) [Q13] [Q15]
- Pneumococcal 20-valent conjugate (PCV20) [Q13] [Q15]
- Pneumococcal 23-valent polysaccharide (PPSV23) [Q13] [Q15]
- Polio (inactivated) [Q13]
- Respiratory syncytial virus (RSV) [Q13]
- Rotavirus [Q13]
- Tetanus booster [Q13]
- Tetanus, diphtheria, pertussis (Td/Tdap) [Q13]
- Varicella [Q13]
- Zoster [Q13]
- Other, specify: _____ [Q13]
- None, because: _____ [P21]
- Unknown [P21]

Q13. Patient: Are you up to date with the vaccinations you need (for age, booster status, or medical condition)? **QM**

- Yes
- No, because: _____ [Q16] [P21]
- Unknown [Q16] [P21]

Q14. Patient: When was your last flu shot? **QM**

- In the past year
- More than a year ago [Q16] [P21]
- Never [Q16] [P21]
- Unknown [Q16] [P21]

Q15. Patient: When was your last pneumonia shot?

- In the last year
- 2 to 4 years ago
- 4 to 5 years ago [Q16] [P21]
- More than 5 years ago [Q16] [P21]
- Never [Q16] [P21]
- Unknown [Q16]

Q16. Patient: Do you have an appointment scheduled to get caught up on your vaccinations?

- Yes
- No [P21]
- Unknown [P21]

Q17. Patient: Do you have other conditions that might make your dementia worse? (Check all that apply.) **QM**

- Alcohol or substance misuse [P2] [P3] Substance-Related Disorders - Self-Care [CCG](#) Substance Use [CCG](#)
- Alzheimer disease [P2] [P3]
- Anxiety [P2] [P3] Anxiety - Self-Care [CCG](#)
- Atrial fibrillation [P2] [P3] Atrial Fibrillation - Self-Care [CCG](#)
- Chronic obstructive pulmonary disease [P2] [P3] Chronic Obstructive Pulmonary Disease - Self-Care [CCG](#)
- Depression [P2] [P3] Depression - Self-Care [CCG](#)
- Diabetes [P2] [P3] Diabetes, Adult - Self-Care [CCG](#)
- HIV/AIDS [P2] [P3] HIV/AIDS - Self-Care [CCG](#)
- Huntington disease [P2] [P3]
- High blood pressure [P2] [P3] Hypertension - Self-Care [CCG](#)
- High cholesterol [P2] [P3] High Cholesterol - Self-Care [CCG](#)
- Multiple sclerosis [P2] [P3] Multiple Sclerosis - Self-Care [CCG](#)
- Obesity or overweight [P2] [P3] Obesity, Adult - Self-Care [CCG](#)
- Osteoporosis [P2] [P3] Osteoporosis - Self-Care [CCG](#)
- Parkinson disease [P2] [P3] Parkinson Disease - Self-Care [CCG](#)
- Physical inactivity [P2] [P3] Exercise [CCG](#)
- Pressure injury [P2] [P3] Wound Management [CCG](#) Pressure Injury Monitoring and Management [CCG](#)

- Posttraumatic stress disorder (PTSD) [P2] [P3] Anxiety - Self-Care  CCG
- Seizures [P2] [P3] Seizure Disorders - Self-Care  CCG
- Stroke [P2] [P3] Stroke - Self-Care  CCG
- Other, specify: _____ [P2] [P3]
- No
- Unknown [P1]

Patient Understanding Evaluation

Q1. Case Manager: Do the patient and caregiver understand all the causes of dementia?

- Yes
- No [P1]
- Unknown [P1]

Q2. Case Manager: Do the patient and caregiver understand all the symptoms of dementia?

- Yes
- No [P1]
- Unknown [P1]

Q3. Case Manager: Do the patient and caregiver understand how to care for dementia? **QM**

- Yes
- No [P1] [P2]
- Unknown [P1] [P2]

Q4. Case Manager: Do the patient and caregiver know when and where to get help for problems?

- Yes
- No [P1] [P3]
- Unknown [P1] [P3]

Q5. Case Manager: Do the patient and caregiver have the ability to understand and follow the self-care plan? **QM**

- Yes
- No [P3]
- Unknown [P3]

Q6. Case Manager: Do the patient and caregiver understand the purpose and side effects of the patient's dementia medication?

- Yes
- No [P5]
- Unknown [P5]

Q7. Case Manager: Do the patient and caregiver have trouble coping with the patient's dementia? **QM**

- Yes [P3]
- No
- Unknown [P3]

Q8. Case Manager: Do the patient and caregiver agree to education? **QM**

- Yes
- No, explain why: _____ [P3] Readiness to Change  CCG
- Unknown [P3]

Quality Measures

- Disease Management Quality Measures:
 - Information provided from this question may be requested by various accrediting organizations.
- HEDIS Measures include(3):
 - Tobacco use is assessed and addressed.
 - Readmission risk is assessed and addressed.
 - Alcohol and substance misuse is assessed and addressed.

Plan of Care

Barriers

B1. Cognitive impairment present

Problems

P1. Dementia information needed

- **Goal:** Dementia diagnosis better understood(4)(5) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **EDUCATE: Definition:** Dementia is a progressive and degenerative central nervous system disorder characterized by cognitive decline.(6)(7)
 - Dementia is associated with a progressive loss of functional skills in daily living, affecting memory, thinking, language, judgment, and behavior.
 - Depression may produce a rapid onset of cognitive deficits; therefore, dementia may present as depression, or it may co-occur with depression.(8)
 - **EDUCATE:** Dementia types(9):
 - Alzheimer disease (AD)
 - Vascular dementia (VaD)
 - Dementia with Lewy bodies (DLB)
 - Other neurodegenerative dementia
 - **EDUCATE: Causes(8):**
 - Alcoholism
 - Alzheimer disease
 - Brain tumors
 - Depression (modifiable risk factor)
 - Head injuries
 - Hydrocephalus
 - Infection: neurosyphilis, HIV
 - Medications (modifiable risk factor): anticholinergics, barbiturates, bromides, glucocorticoids, haloperidol, opiates
 - Metals: lead, mercury
 - Vitamin deficiency: B12, thiamine
 - **EDUCATE: Symptoms(7)(9):**
 - Emotional affect alteration (eg, flat, inappropriate, blunt, labile)
 - Cognitive changes (eg, confusion, memory loss)
 - Depression
 - Agitation
 - Concentration difficulty
 - Gait, motor, and balance problems
 - Hallucinations
 - Hygiene poor
 - Impulse control problems
 - Thought process problems (eg, disorganized speech, trouble completing activities, concentration difficulty)
 - Delusions (eg, paranoid)
 - **EDUCATE: Diagnosis^[D](7):**
 - Based on medical history, risk factors, physical examination, and diagnostic tests
 - Behavioral health examination and history
 - Positive screening instrument
 - **EDUCATE: Risk factors(6)(10):**
 - Varies by type and age of onset; result of multiple factors rather than a single cause
 - Age is a significant factor for Alzheimer disease, with incidence doubling after age 65 years.
 - Cardiovascular disease and factors that increase risk of cardiovascular disease (ie, smoking, obesity, hypertension)
 - First-degree relative with Alzheimer disease or genetic predisposition
 - Long-term neurologic conditions such as Parkinson disease
 - Traumatic brain injury
 - **EDUCATE: Treatment(8)(11):**
 - ADL and IADL support
 - Behavioral health therapy options
 - Comorbid condition management
 - End-of-life or palliative care
 - Medication management
 - Medications for pre-existing conditions may need to be changed (type or dose).(12)

- No cure
- Physical, occupational, or speech therapy evaluation and treatment, if indicated
- Self-care activities (eg, diet, activity, medication)(6)
- Symptom management
- **EDUCATE: Treatment goals(6):**
 - Advance care planning
 - Complication and progression management
 - Educate and support caregivers
 - Functional improvement and maintenance (eg, ADL, IADL)
 - Psychosocial functioning improved and maintained
 - Quality of life improvement
 - Safety (for self and others)
 - Symptom frequency and severity reduced
- **EDUCATE: Resources:**
 - Alzheimer's Association: www.alz.org
 - Dementia.org: www.dementia.org
 - Smokefree. National Institutes of Health: www.smokefree.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated(13)
- **COORDINATE:** Neurologist referral, if indicated(14)
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Social worker referral, as needed(6)
- **COORDINATE:** Laboratory testing, if indicated(7)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up to evaluate dementia understanding, referral status, and need for further assistance(19)(20) 
- **SEND:** Your care plan - Dementia  (Spanish)
- **SEND:** What you need to know - Dementia  (Spanish)
- **SEND:** When to get help - Dementia  (Spanish)
- **SEND:** Daily care for your condition - Dementia  (Spanish)
- **SEND:** Be ready for your doctor visit - Dementia  (Spanish)
- **SEND:** Be ready with questions for your doctor - Dementia  (Spanish)

P2. Dementia self-care needed

- **Goal:** Dementia self-care plan developed, implemented, and maintained(4)(21)(22) 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Advance care planning(14)
 - **ASSIST:** Obtaining a medical alert bracelet or necklace
 - **ASSIST:** Communication between multiple providers(23)
 - **EDUCATE: Self-care plan information:**
 - Activity, regular pattern(24)
 - Daily care (eg, keep list of important tasks, organize your things)
 - Break down all tasks into simple steps.
 - Orientation management (eg, provide photographs, calendars, clocks; use dentures, glasses, hearing aids; arrange activities with family and friends)
 - Symptom management (eg, agitation, aggression, wandering)(9)
 - Trigger avoidance (eg, high-stress situations, sleep deprivation)
 - Caregiver support plan, as needed(14)(25)
 - Crisis care plan: turn off distractions (eg, TV, radio), seek emergent care, call hotline, call police if escalates; caregivers advised to not argue
 - De-escalation techniques (eg, caregiver remains calm and provides emotional support, do not argue or threaten, stay serious, announce actions beforehand)(26)
 - Medication purpose, use, effects, and considerations(27)
 - Diet considerations (eg, tube feedings, dysphagia diet)(28)
 - Skin check routine, assessing high-risk pressure injury areas(29)

- Bowel and bladder program(30)
- Sleep hygiene (eg, regular pattern, calm sleep environment)(7)
- Structured activities (eg, exercise therapy, pet therapy, social contact)(11)(31)
- Exercise program within provider's recommendations[E](32)(33)[N]
- Functional changes care (eg, adaptations and support for ADL and IADL)(34)
- Medical equipment use and care
- Medical alert bracelet/necklace, obtain and wear
- Safety awareness and management (eg, burn prevention, fall prevention)(35)(36)
- Wellness checkups (eg, vaccinations, screenings)
- Comorbid condition management[F](8)
- Pain management plan(37)(38)
- Stress management (eg, relaxation and coping techniques)
- Support group participation(6)
- Driving restrictions, as needed(38)(39)
- Advance care planning(6)(14)
- Tobacco cessation plan(40)
- **EDUCATE: When to get emergency help(24):**
 - Suicidal or homicidal actions or plans(41)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(42)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(43)
 - Crisis plan not working
 - Delirium (eg, reduced awareness, agitation)(38)
 - Psychotic behavior (eg, threatening)(38)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(44)
 - Consciousness level change, abrupt
 - Hallucinations or delusions, increased or new
 - Seizures, new or complicated (eg, seizure longer than 5 minutes, multiple seizures with loss of consciousness, head injury during seizure)(45)
- **EDUCATE: When to call the provider for behavior changes(46):**
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)(47)
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(26)(48)
 - Self-care changes (eg, ADL or IADL problems)(49)
 - Social isolation
 - Appointments missed (eg, support group, provider)
 - Sleep pattern changes (eg, hypersomnia, insomnia)
 - Mood changes (eg, depression, anger, agitation)(47)
 - Fatigue, unrelenting
- **EDUCATE: When to call the provider(8)(50):**
 - Caregiver cannot take care of patient anymore.(39)
 - Weight gain or loss without reason
 - Constipation
 - Diarrhea for more than 6 hours
 - Medication problems (eg, side effects, access, administration)
 - Medications newly ordered from other providers
 - Medical equipment problems or needs
 - Dizziness or balance problems
 - Malnutrition (eg, weight loss, dehydration, muscle loss)(28)(47)
 - Falls(46)
 - Fever, prolonged or unresponsive to treatment(46)
 - Blood glucose outside of provider's parameters
 - Physical activity level declines (eg, fatigue, weakness).
 - Pain changes (eg, new or worsening, unresolved with treatment)(47)
 - Cold or flu symptoms that do not go away
 - Skin problems (eg, rash, nonhealing sore)
 - Bowel or bladder dysfunction
- **COORDINATE: End-of-life or palliative care referral, if indicated(14)(51)(52)**
- **COORDINATE: Community services referral, as needed(39)(53)**
- **COORDINATE: Rehabilitation therapy referral, if indicated(54)(55)**
- **COORDINATE: Support group referral, as needed**

- **COORDINATE:** Respite care referral, as needed(38)
- **COORDINATE:** Medical equipment and supplies, if indicated
- **COORDINATE:** Tobacco use treatment, as needed
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
- **SEND:** Your care plan - Dementia  (Spanish)
- **SEND:** What you need to know - Dementia  (Spanish)
- **SEND:** When to get help - Dementia  (Spanish)
- **SEND:** Be ready for your doctor visit - Dementia  (Spanish)
- **SEND:** Be ready with questions for your doctor - Dementia  (Spanish)
- **SEND:** Daily care for your condition - Dementia  (Spanish)
- **SEND:** Caregiver tips  (Spanish)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Bladder care  (Spanish)
- **SEND:** Constipation care  (Spanish)
- **SEND:** Helping people who cannot think clearly  (Spanish)
- **SEND:** Help with sleeping or insomnia  (Spanish)
- **SEND:** How to manage your pain  (Spanish)
- **SEND:** How to stop smoking  (Spanish)
- **SEND:** How to use a cane (Illustrated)  (Spanish)
- **SEND:** How to use a walker (Illustrated)  (Spanish)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** Get ready to use a wheelchair at home  (Spanish)
- **SEND:** Prevent leg ulcers  (Spanish)
- **SEND:** Preventing pressure injuries (Illustrated)  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Safety in the home - Fire and other risks  (Spanish)
- **SEND:** Safety in the home - Memory loss  (Spanish)
- **Goal:** Behavioral health therapy initiated and maintained
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of behavioral health status
 - **ASSIST:** Developing and implementing a plan to address depression and cognitive and behavioral deficits(46)
 - **ASSIST:** Communication between multiple providers
 - **EDUCATE:** Make and keep all appointments with psychiatrist, psychologist, therapist, or social worker to help address disease symptoms and provide a supervised program.
 - **EDUCATE:** Factors that may affect quality of life (eg, relationships, agency in life today, wellness perspective, and sense of place)
 - **EDUCATE:** Medical conditions that contribute to psychiatric symptoms (eg, pain, constipation, dehydration, infection, visual and hearing impairment, medication side effects, and substance use)(46)
 - **EDUCATE:** Behavioral health monitoring and care (eg, depression monitoring and management)(46)(51)
 - **EDUCATE:** When and how to get help for depression, memory loss, or psychosis
 - **EDUCATE:** Guided adjustment of dementia self-care plan as indicated by provider. Treatment options:
 - Medication management
 - Behavioral health treatment (eg, cognitive therapy, family therapy, music therapy)(46)(56) **NNN**
 - **EDUCATE:** Agitation management(7)(46):
 - Keep tasks and their explanations simple.
 - Velcro for clothing and shoes to simplify
 - Keep consistent routine.
 - Stay at arm's length to avoid startling.

- **EDUCATE:** Wandering interventions(7)(46):
 - Post signs with information and instructions.
 - Safety alarms for falls, fire, doors
 - Install difficult key latches.
 - Give patient ID bracelets, clothing tags, or tracking device.
- **EDUCATE:** Cognitive strategies:
 - Focus on process instead of results.
 - Break down all tasks into simple steps.
 - Encourage participation in ADL and IADL, but do not force.
 - ADL/IADL scheduled at times during the day when better able to perform or participate
 - Memory exercises to promote retention(57)
- **COORDINATE:** Behavioral health services referral, if indicated(58)
- **COORDINATE:** Psychiatrist referral, if indicated(51)
- **COORDINATE:** Social worker referral, as needed(11)(59)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient and caregiver to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
- **SEND:** Be ready for your doctor visit - Dementia  (Spanish)
- **SEND:** Be ready with questions for your doctor - Dementia  (Spanish)
- **SEND:** Be ready for your doctor visit - Depression  (Spanish)
- **SEND:** Be ready with questions for your doctor - Depression  (Spanish)
- **SEND:** When to get help - Dementia  (Spanish)
- **SEND:** When to get help - Depression  (Spanish)
- **SEND:** When to get help - Substance-related disorders  (Spanish)
- **Goal:** Nutritional status optimized
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **EDUCATE:** Dietary recommendations to support optimal weight and nutrient intake:
 - Diet considerations (eg, Mediterranean-style diet) (11)
 - Diet supplements to increase caloric and vitamin intake and decrease potential for osteoporosis (calcium and vitamin D)(60)(61)
 - Weight monitoring and management
 - Consider food amount and texture to enhance swallowing ability, per speech-language pathologist (SLP) recommendations.
 - Aspiration prevention precautions(62)
 - Dysphagia management, as needed(7)
 - Nutritional therapy
 - **COORDINATE:** Dietitian referral, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated(54)(55)
 - **COORDINATE:** Home care referral, if indicated(13)
 - **COORDINATE:** Gastroenterologist referral, if indicated
 - **COORDINATE:** Care team meeting, if needed(15)(16)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact with patient and caregiver to evaluate understanding and adherence to self-care plan, referral status, and need for further assistance(19)(20)
 - **SEND:** Eating when it is hard to swallow  (Spanish)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** PEG tube care - Giving medicine (Illustrated)  (Spanish)
 - **SEND:** PEG tube care - Giving a feeding (Illustrated)  (Spanish)

P3. Dementia potential complications and comorbidities monitoring needed

- **Goal:** Dementia potential complications and comorbidities evaluated and addressed(4)

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling for evaluation of problems, comorbidities, and self-care plan^[H]:
 - Recently hospitalized patient's evaluation within 1 week of discharge^N
 - Recently hospitalized patient's evaluation with outpatient healthcare providers (eg, nurse, social worker) within 1 to 3 days of discharge⁽¹⁹⁾
 - Appointment every 3 to 6 months when patient is symptom free
 - Appointment once a month or more for patient with questionable adherence to treatment plan
- **EDUCATE:** Adherence to plan necessary to achieve goals, and complications of nonadherence (eg, exacerbations, progression, comorbidities)
- **EDUCATE:** Common complications include falls, fractures, osteoporosis risk, overnutrition, undernutrition, dysphagia, dehydration, and pressure injuries.⁽⁷⁾⁽⁶¹⁾
- **EDUCATE:** When and how to get help for worsening dementia problems
- **EDUCATE:** Treatment and monitoring for comorbid conditions (eg, coronary artery disease, high blood pressure, heart valve disease)
- **EDUCATE:** Follow-up appointments for assessment of clinical status
- **COORDINATE:** Specialist referral, if indicated
- **COORDINATE:** Treatment program referral, if indicated
- **COORDINATE:** Care team meeting, if needed⁽¹⁵⁾⁽¹⁶⁾
- **COORDINATE:** Communication between care team members and care plan reconciliation⁽¹⁵⁾⁽¹⁷⁾⁽¹⁸⁾
- **COORDINATE:** Follow-up contact to review self-care plan and need for further assistance⁽¹⁹⁾⁽²⁰⁾
- **SEND:** Be ready for your doctor visit - Dementia  (Spanish)
- **SEND:** Be ready with questions for your doctor - Dementia  (Spanish)

P4. Dementia medication evaluation needed

- **Goal:** Dementia medication evaluation completed and medication regimen initiated, if indicated

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for evaluation of condition status
- **EDUCATE:** When and how to get help for worsening dementia problems
- **EDUCATE:** Medication management needs common for dementia
- **COORDINATE:** Follow-up after provider contact for evaluation of medication changes⁽¹⁹⁾⁽²⁰⁾

P5. Dementia medication management needed

- **Goal:** Medication information understood⁽⁶³⁾

- o **Interventions:**

- **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for discussion on appropriate medication
- **ASSIST:** Providing reconciled medication list to all providers. See Medication Reconciliation Tool  SR for more information.
- **EDUCATE: Purpose:** Importance of understanding what each medication is for, how it should be taken, and what to do if dose is missed
- **EDUCATE:** Dementia medications purpose and dosing schedule; goals include⁽⁶⁴⁾:
 - Slow cognitive deterioration (time to death or nursing home placement)
 - Preserve cognitive and functional ability
 - Minimize behavioral disturbance
- **EDUCATE:** Medication indications, side effects, and considerations frequently change. See a drug reference for specific information not listed here.
- **EDUCATE:** Administration techniques:
 - Route (eg, oral, intramuscular (IM), intravenous (IV))
 - With or without foods or liquids (eg, milk)
 - Timing of administration
 - Appropriate measuring device (eg, liquid medications)

- Proper storage
- **EDUCATE:** Caregiver's medication administration information(65):
 - Supervise patient taking medication.
 - Remove medication from sight once taken.
 - Maintain safety.
 - Monitor side effects.
 - Report problems to healthcare provider.
 - Establish backup plan to maintain continuity of medication regimen.
 - Transdermal patch management - practice good skin-washing technique after removal of transdermal patch to prevent overdosing.
- **EDUCATE: Get emergency help:** Adverse or allergic reactions (eg, wheezing, chest tightness, fever, itching, bad cough, blue skin color, hemorrhage, seizures, swelling of face, lips, tongue, or throat)
- **EDUCATE: Side effects:** Management, and when to contact provider
 - Common side effects include gastrointestinal changes, dry mouth, fatigue, insomnia, rash, dizziness, and headache.
- **EDUCATE: Considerations:**
 - Discuss medication with provider if pregnant, planning a pregnancy, or breast-feeding.
 - Comorbidities may be contraindications to the use of a drug (eg, liver disease, diabetes).
 - Health checks, diary, and action plans (eg, blood pressure, weight, heart rate)
 - Safety measures (eg, sun protection, driving, or operating machinery)
 - Contact provider or pharmacist prior to taking any other over-the-counter or prescribed medication.
 - Continue taking medication even if feeling better.(66)
- **COORDINATE:** Clinical monitoring appropriate to medication use (eg, laboratory tests)
- **COORDINATE:** Communication between patient and pharmacy (for obtaining prescriptions, setting up home medication delivery, review for drug-drug interactions)
- **COORDINATE:** Communication between care team members and medication list reconciliation
- **COORDINATE:** Follow-up to evaluate adherence or problems with medication, provide additional education and support, and identify need for further assistance(19)(20)
- **COORDINATE:** Laboratory testing, if indicated
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated
- **COORDINATE:** Therapeutic drug level monitoring
- **SEND:** Managing your medicines (Illustrated)  (Spanish)

P6. Antidepressant medication management needed

- **Goal:** Antidepressant medication initiated and maintained(26)(48)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Antidepressants may be indicated to help relieve symptoms of depression.
 - Effects may take 2 to 8 weeks.
 - Most people need to be on medication at least 18 to 24 months after adequate response to symptoms.
 - **EDUCATE: Call provider immediately:** Suicidal ideation
 - **EDUCATE: Side effects:** Sedation, dry mouth
 - **EDUCATE: Considerations:**
 - Augmentation agent may be added or change in antidepressant may occur if symptoms do not improve.
 - Withdrawal symptoms may occur when discontinued.
 - **SEND:** Antidepressant medicines  (Spanish)

P7. Cholinesterase inhibitor medication management needed

- **Goal:** Cholinesterase inhibitor medication initiated and maintained(67)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Cholinesterase inhibitors are used to treat mild to moderate Alzheimer dementia.(39)
 - **EDUCATE: Call provider immediately:** Sore throat or mouth sores
 - **EDUCATE: Side effects:** Dry mouth, orthostatic hypotension, heat intolerance
 - **EDUCATE: Considerations:** Use good oral hygiene, change positions slowly, avoid high environmental heat (eg, hot tubs, hot weather), and use sun protection measures.

P8. Memantine medication management needed

- **Goal:** Memantine initiated and maintained(39)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Memantine is used to treat moderate to severe Alzheimer dementia.

- **EDUCATE: Side effects:** Hypertension, pain, fatigue

P9. Typical antipsychotic medication management needed

- **Goal:** Typical antipsychotic medication initiated and maintained(68)(NN)
- **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** Antipsychotics may be indicated for agitation, aggression, or violence.(I)(69)(70)
 - Effects may take several weeks.
 - **EDUCATE: Call provider immediately:** Neuroleptic malignant syndrome (NMS) (eg, extreme muscle rigidity, high fever, tachycardia, tachypnea, diaphoresis, stupor, fluctuations in blood pressure)(J)(71)
 - **EDUCATE: Side effects:** Drowsiness, dry mouth, weight gain, ECG changes, involuntary reflexes and movements, hyperglycemia, diabetes
 - Tardive dyskinesia (involuntary repetitive movements, such as grimacing, tongue movements, lip smacking, pursed lips, excessive eye blinking) can occur.(K)
 - Increased risk of metabolic side effects, including obesity, diabetes, and dyslipidemia
 - **EDUCATE: Considerations:**
 - PRN medications may be ordered to address involuntary reflexes and movements.
 - Monitor blood glucose, cholesterol, and triglyceride levels.
 - Diet and exercise instruction
 - Modest efficacy and increases risk of cognitive decline and cerebrovascular events(L)(72)
 - Talk to provider before discontinuing medication.
 - **COORDINATE:** Electrocardiogram (ECG) monitoring, if indicated

P10. Fall risk evaluation needed

- **Goal:** Falls prevented and fall management plan followed
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for medical evaluation of functional changes and creation of self-care plan
 - **ASSIST:** Creation of rehabilitation treatment plan between patient and care team (therapists and other providers)(N)
 - **EDUCATE:** Fall risk identification(73)(74):
 - Age
 - Female
 - Low body mass
 - Behavioral health comorbidities
 - Musculoskeletal diseases
 - Cognitive impairment
 - Gait and balance problems
 - History of falls
 - Improper footwear (eg, backless shoes or slippers, shoes without nonskid soles, high-heeled shoes, or smooth leather soles)
 - Sensory impairments
 - Orthostatic hypotension episodes
 - Medications: antiarrhythmics, antidepressants, antihypertensives, antiseizure medications, benzodiazepines, and diuretics(75)(76)(77)
 - Pain
 - Medical comorbidities
 - **EDUCATE:** Home changes to help prevent falls:
 - Remove all throw rugs and clutter from floors.
 - Repair carpet tears, broken tiles, or hardwood floor ruts that could cause tripping.
 - Clear walking areas of electrical cords or wires.
 - Move furniture to allow for use of crutches or walker.
 - Keep all areas well lit.(78)
 - Clean up spills immediately.
 - **EDUCATE:** Self-care safety activities(62)(79):
 - Rise slowly from lying or sitting position.
 - Wear glasses or contact lenses, if needed.
 - Use handrails, grab bars, or adaptive devices as needed.
 - Wear shoes with nonslip soles.
 - Use no-slip mats in shower or tub.
 - Pet-related falls mitigation (eg, not chasing pet, obedience training)

- Know which medication may cause problems (eg, dizziness, weakness, or balance problems).(80)
- Exercise program within provider's recommendations(32)(81)
- **EDUCATE: When to get emergency help:**
 - Fall complications (eg, head injury, broken bone, significant ADL changes)
 - Fear of personal harm from someone (eg, spouse, parent).(82) See Intimate Partner Violence  CCG.
 - Stroke, heart attack, or blood clot
- **EDUCATE: When to call the provider:**
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Physical activity level declines (eg, fatigue, weakness).
 - Depression (eg, feeling sad, hopeless, overwhelmed)(26)(48)
 - Medication problems (eg, side effects, access, administration)
 - Self-care changes (eg, ADL or IADL problems)
 - Falls
- **COORDINATE:** Pharmacy medication support, if indicated[M](73)
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Rehabilitation therapy referral, if indicated(83)(84)
- **COORDINATE:** Abuse or violence advocacy, as needed
- **COORDINATE:** Medical equipment and supplies, if indicated(85)(86)
- **COORDINATE:** Community services referral, as needed[N](87)
- **COORDINATE:** Social worker referral, as needed(59)
- **COORDINATE:** Home care referral, if indicated(84)(88)
- **COORDINATE:** Outpatient imaging, tests, or procedures, if indicated(89)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate fall risk, referral status, and need for further assistance(19)(20)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** How to get out of bed safely (Illustrated)  (Spanish)
- **SEND:** How to use a walker (Illustrated)  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Ways to save energy  (Spanish)
- **SEND:** Get ready to use a wheelchair at home  (Spanish)
- **SEND:** How to use a cane (Illustrated)  (Spanish)

P11. Physical function impairments present

- **Goal:** Physical functioning improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for medical evaluation of functional changes and creation of self-care plan
 - **ASSIST:** Creation of rehabilitation treatment plan between patient and care team (therapists and other providers)(90)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **EDUCATE:** Ways to manage physical impairments with self-care plan(85):
 - Importance of physical activity to maintain full functional level
 - Value of rehabilitation(83)
 - Use of assistive devices
 - Exercise recommendations
 - Home environmental safety
 - Transportation safety: modified van with lift, sensitized hand controls, hand controls
 - **EDUCATE:** When and how to get help for problems:
 - Fear of personal harm from someone (eg, spouse, parent).(82) See Intimate Partner Violence  CCG.
 - When to get emergency help because of ADL problems (eg, fall with loss of consciousness, stroke or heart attack signs, lack of oxygen)
 - When to call the provider for ADL problems (eg, needing more help to perform, change of condition)

- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Rehabilitation therapy referral, if indicated(90)
- **COORDINATE:** Medical equipment and supplies, if indicated(85)
- **COORDINATE:** Community services referral, as needed(87)
- **COORDINATE:** Social worker referral, as needed(59)
- **COORDINATE:** Home care referral, if indicated(88)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact with patient to evaluate physical impairments, referral status, and need for further assistance(19)(20)
- **SEND:** Choosing a place to live  (Spanish)
- **SEND:** How to move safely in bed (Illustrated)  (Spanish)
- **SEND:** Exercising to improve balance (Illustrated)  (Spanish)
- **SEND:** Exercising when you have neurologic problems  (Spanish)
- **SEND:** Safety in the home - Fall protection  (Spanish)
- **SEND:** Get ready to use a wheelchair at home  (Spanish)
- **SEND:** Ways to save energy  (Spanish)

P12. Aspiration risk evaluation needed

- **Goal:** Aspiration risk evaluated and management plan initiated(91)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Self-care plan creation between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **EDUCATE:** Reducing aspiration risk and complications(77)(92):
 - Frequent small meals and small bites when eating
 - If patient has one-sided facial weakness, place food on other side of mouth.
 - Oral hygiene before meals to stimulate taste
 - Postpone meal if fatigued.
 - Avoiding excessive sedation during mealtimes
 - Upright position for feedings and medications. Keep upright for 30 minutes after meals.
 - Reduce distractions.
 - Alternate solids with fluids while eating.
 - Caregivers should feed patient slowly and monitor patient for pocketing food, aspiration, or choking.
 - Training in Heimlich maneuver and CPR for caregivers
 - Consider size, shape, taste, and texture of medications. Alternatives may be needed (eg, liquid, modified-release capsules, or enteric-coated tablets or capsules).
 - **EDUCATE:** Meal planning(93):
 - Selecting foods within care team recommendation (eg, mashed potatoes for soft foods)
 - Food preparation ideas based on diet recommendations
 - **EDUCATE:** Ways to improve swallowing ability(94):
 - Change in body position
 - Swallowing exercises
 - Avoid difficult-to-swallow foods (eg, peanut butter or caramel).
 - **EDUCATE:** Feeding techniques(93):
 - Give small bites, or make sure patient takes small bites.
 - Feed slowly, or make sure patient eats slowly. Alternate solids and liquids while feeding.
 - Check for food in patient's mouth before giving additional bites.
 - **EDUCATE: When to get emergency help(77)(94):**
 - Aspiration (eg, coughing, choking, hypoxia, wheezing)
 - Respiratory impairment (eg, gasping for air, choking, cyanosis, chest pain, severe dyspnea)(95)
 - Trouble talking
 - **EDUCATE: When to call the provider(77)(96):**
 - Swallowing changes (eg, pain, difficulty, new-onset choking or coughing)
 - Drooling when eating or drinking
 - Food comes up when you try to swallow.
 - Takes more than 2 swallows to clear food from throat

- **COORDINATE:** Speech-language pathologist referral, if indicated(97)
- **COORDINATE:** Dietitian referral, if indicated
- **COORDINATE:** Medical equipment and supplies, if indicated(92)
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Pharmacy medication support, if indicated(77)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up for evaluation of aspiration, status of referrals, and need for further assistance(19)(20)
- **SEND:** Eating when it is hard to swallow  (Spanish)

P13. Sleeping problems present

- **Goal:** Sleep improved 
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Scheduling provider appointment for medical evaluation of sleep changes and creation of self-care plan to include(80)(98):
 - Identifying techniques to improve sleep quality(99)
 - Medication changes that can help patient sleep better (eg, changing the time stimulants are taken to early in the day or not taking diuretic before bedtime)
 - Review of herbal preparations that may promote sleep(100)
 - Creation of sleep diary(101)
 - **EDUCATE:** Sleep hygiene techniques(80)(99)(102):
 - Schedule sleep (go to bed and get up at same time every day).
 - Spend no more than 8 hours per day in bed. Amount of average sleep time for adult is 7 to 8 hours.
 - Limit napping to once a day for 20 to 30 minutes in early afternoon.
 - Sleep in a dark, quiet bedroom.
 - Use bedroom only for sleeping, not to watch TV, work, use electronics, or do other activities.
 - Avoid pets in the bed.
 - Get safe amount of sunlight every day and avoid bright lights before bedtime.
 - Cut down on drinking fluids in the afternoon if nighttime urination is a problem.
 - Avoid substances that may influence sleep (eg, caffeine, stimulants, alcohol).
 - Stop smoking; nicotine is a stimulant.
 - Get regular exercise, but avoid vigorous exercise 2 hours before bedtime.(103)
 - Write down urgent thoughts if mind-racing is a problem.
 - Do not eat big meals before bedtime; gastric process can cause sleep problems.
 - **EDUCATE:** Herbal preparations to help sleep (eg, melatonin, tryptophan, valerian), as approved by provider(104) 
 - **EDUCATE:** Behavioral health treatment options and benefits (eg, cognitive behavioral therapy for insomnia)(101)(105) 
 - **EDUCATE:** Hypnotic medications use and misuse, if prescribed for insomnia(98)
 - **COORDINATE:** Sleep specialist referral, if indicated(104)
 - **COORDINATE:** Behavioral health services referral, if indicated(101)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to evaluate sleep, status of referrals, and need for further assistance(19)(20)
 - **SEND:** Help with sleeping or insomnia  (Spanish)

P14. Cognitive impairment present

- **Goal:** Cognitive and behavioral symptoms addressed and managed(38)(106)
- **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Support system (eg, family, friends) identification, enlistment, and education
 - **ASSIST:** Appointment scheduling for evaluation of new or worsening change in behavioral or cognitive function, and the development of a self-care plan
 - **ASSIST:** Creation of a self-care plan between patient and provider (eg, goals, management, treating symptoms, getting help)
 - **ASSIST:** Communication between multiple providers(23)
 - **EDUCATE:** Cognition information(39):
 - Cognition is ability to learn, solve problems, remember, and appropriately use information.

- Problems or deficits are part of aging process, but specific disorders may be treatable (eg, delirium, dementia, depression).
- Cognition deficits may be progressive.
- When and how to get help if cognition problems progress to point caregiver cannot provide care
- **EDUCATE:** Ways to help manage cognitive changes(6)(39):
 - Agitation management (eg, simplify tasks and instructions, keep routine consistent)
 - Maintain a calm environment.
 - Set small goals every day (eg, get out of bed at a certain time, finish one chore or project, or do something that brings pleasure, such as listening to music).
 - Structured activities (eg, exercise therapy, pet therapy, social contact)
 - Focus on process instead of results.
 - Break down all tasks into simple steps.
 - ADL/IADL scheduled at times during the day when better able to perform or participate
 - Encourage participation in ADL and IADL, but do not force.
 - Organizational strategies and problem-solving tasks, including note-taking devices, cueing, and using memory books
 - Mentally stimulating activities participation (eg, reading, puzzles)
 - Orientation aids (eg, calendars, clocks, and alarms)
- **EDUCATE:** Medication side effects for cognitive and behavioral deficits (eg, antipsychotics, tricyclic antidepressants, SSRI antidepressants, dementia drugs)
- **EDUCATE:** Cognitive deficits may be progressive, and how to get help if they progress to point caregiver cannot provide care.
- **EDUCATE: When to get emergency help:**
 - Suicidal or homicidal actions or plans(26)
 - Psychotic behavior (eg, threatening)
 - Heart attack (eg, persistent chest pain; pain radiating to the arms, shoulders, neck, back, and/or jaw; cool clammy skin)(42)
 - Stroke (eg, face drooping, arm weakness, speech problems, bad headache, vision changes, loss of balance or coordination)(43)
- **EDUCATE: When to call the provider(6):**
 - Depression (eg, feeling sad, hopeless, overwhelmed)(26)
 - Hallucinations or delusions increase.
 - Cognitive changes (eg, thinking problems, forgetfulness worsens)
 - Insomnia
 - Self-care changes (eg, ADL or IADL problems)
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Home care referral, if indicated
- **COORDINATE:** Neurologist referral, if indicated
- **COORDINATE:** Psychiatrist referral, if indicated
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Social worker referral, as needed(59)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Rehabilitation therapy referral, if indicated(38)
- **COORDINATE:** Caregiver health service referral, as needed
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up evaluation for problems due to cognitive impairment, referral status, and need for further assistance(19)(20)
- **SEND:** Caregiver tips 📄 (Spanish)
- **SEND:** Helping people who cannot think clearly 📄 (Spanish)
- **SEND:** Safety in the home - Memory loss 📄 (Spanish)
- **SEND:** Help with sleeping or insomnia 📄 (Spanish)
- **SEND:** Tips for good mental health 📄 (Spanish)
- **SEND:** Recognizing the signs of depression 📄 (Spanish)

P15. Healthy diet information needed

- **Goal:** Healthy diet better understood(107)(108)📄
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up

- **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
- **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
- **EDUCATE:** Healthy body weight and body mass index (BMI) for patient(109):
 - BMI equals body weight in kilograms divided by the square of the height in meters (kg/m²).
 - BMI of 18.5 to 24.9 is a normal weight.
 - BMI of 25 to 29.9 is overweight.
 - BMI of 30 or greater is obese.
- **EDUCATE:** Healthy diet general recommendations(110)(111):
 - Well-balanced diet (ie, high in fruits and vegetables, low in sodium, fats, and sugars)
 - Drink enough fluids.
 - Safe eating habits (eg, do not skip meals)
 - Calorie amount appropriate (eg, factors: age, gender, activity level)
- **EDUCATE:** Adequate fluid intake(P)(111)(112):
 - Amount of fluid intake needed depends on patient attributes, including medical condition, level of physical activity, and exposure to heat.[N](#)
 - Men age 19 years and older: 15.6 cups (3.7 L) per day from all sources (eg, food and fluids)
 - Women age 19 years and older: 11.4 cups (2.7 L) per day from all sources (eg, food and fluids)
- **COORDINATE:** Dietitian referral, if indicated(113)
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Follow-up contact with patient to evaluate diet understanding and adherence, referral status, and need for further assistance(19)(20)
- **SEND:** Healthy eating (Illustrated)  (Spanish)
- **SEND:** How to read food labels (Illustrated)  (Spanish)
- **SEND:** Weight diary (Illustrated)  (Spanish)
- **SEND:** Meal diary (Illustrated)  (Spanish)

P16. Potential nutritional problems

- **Goal:** Nutritional problems resolved(94)(108)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for dietary review and self-care plan
 - **ASSIST:** Creation of nutrition diary
 - **COORDINATE:** Laboratory testing, if indicated
 - **COORDINATE:** Calorie or fluid counting
 - **EDUCATE:** Nutritional recommendations for patient's health status(111)
 - **EDUCATE:** Food and drug interactions potentially impacting nutritional status
 - **COORDINATE:** Dietitian referral, if indicated(113)
 - **COORDINATE:** Home care referral, if indicated
 - **COORDINATE:** Pharmacy medication support, if indicated
 - **COORDINATE:** Rehabilitation therapy referral, if indicated
 - **COORDINATE:** Follow-up contact with patient to evaluate nutritional status, referral status, and need for further assistance(19)(20)
 - **SEND:** Eating to keep up your energy  (Spanish)
 - **SEND:** Eating when your stomach is upset  (Spanish)
 - **SEND:** Mouth sores or infections  (Spanish)
 - **SEND:** How to read food labels (Illustrated)  (Spanish)
 - **SEND:** Meal diary (Illustrated)  (Spanish)

P17. Food label reading information needed

- **Goal:** Food label understanding improved
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)

- **ASSIST:** Appointment scheduling with provider, nurse, or dietitian to provide in-person instruction on how to read food labels
- **EDUCATE:** Food labels contain the nutritional value of the product(114)(115):
 - Percent of daily values is usually based on a 2000-calorie (8374-kJ) diet.
 - Nutritional values are listed per one serving.
 - A package may have more than one serving in the whole package.
 - Nutritional value should be doubled, tripled, and so on based on the number of servings eaten.
 - Serving size varies from product to product.
 - Form of food has impact on nutrition (eg, increased sugar content of orange juice vs one orange).
- **COORDINATE:** Dietitian referral, if indicated
- **COORDINATE:** Follow-up contact to evaluate food label understanding, referral status, and need for further assistance(19)(20)
- **SEND:** How to read food labels (Illustrated)  (Spanish)

P18. Tobacco use present

- **Goal:** Tobacco abstinence achieved(116) **NNN**
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation for nicotine replacement therapy (NRT) or other medications to help patient quit using tobacco
 - **ASSIST:** Developing a plan to quit(117):
 - Setting quit date(118)
 - Identifying reasons to quit (eg, add years to your life, for someone you love, save money, feel better)
 - Identifying techniques to quit (ie, "cold turkey," using nicotine replacement, or gradual withdrawal)(119)
 - Identifying coping mechanisms
 - **ASSIST:** Identifying resources to help tobacco cessation
 - **EDUCATE:** Impact of tobacco on life(120):
 - Current symptoms caused by tobacco use
 - Costs of tobacco use
 - Impact of tobacco use on home environment
 - Risks of continued smoking on health (eg, cancer, stroke, gum disease)
 - **EDUCATE:** Benefits of tobacco cessation:
 - Within minutes to days - lower blood pressure, lower carbon monoxide levels, better stamina, and improved smell and taste
 - Within 2 to 4 weeks - less risk of respiratory infection
 - Within 1 to 3 months - improved lung function and circulation
 - Within 1 year - 50% reduction in risk for myocardial infarction(121)
 - Within 5 to 15 years - cardiovascular risk equal to that of someone who has never smoked
 - Within 10 years - lung cancer risk cut in half
 - **EDUCATE:** Ways to stop smoking or using tobacco(120):
 - Get support (support group, counseling, family buy-in, written self-help materials).(122) **NNNNN**
 - Talk to providers for help.
 - Stress management (eg, relaxation and coping techniques)(118)
 - Nicotine replacement therapy (eg, patch, gum) or medications that may help **Q** **N**
 - Healthy lifestyle (good nutrition, activity)(118)(123) **N**
 - **EDUCATE:** Withdrawal symptoms that may be experienced (eg, trouble sleeping, replacing smoking urge with eating)
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Motivational counseling if patient not ready to quit(124)
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Participation in stop-smoking programs (eg, American Cancer Society, American Lung Association)
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to monitor for withdrawal and evaluate tobacco use, referral status, and need for further assistance(19)(20)
 - **SEND:** How to stop smoking  (Spanish)
 - **Goal:** Tobacco cessation medication treatment initiated and maintained(67) **NN**
 - **Interventions:**

- **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
- **EDUCATE: Purpose:** Medication is used to reduce withdrawal feeling during smoking cessation.
 - Bupropion sustained-release and varenicline are first-line treatment, either alone or in combination.
 - Bupropion is recommended for 7 to 12 weeks. Continued treatment may be needed.
 - Varenicline can be used as maintenance therapy to prevent relapse.(125)
- **EDUCATE: Call provider immediately:** Suicidal ideation, behavioral changes, or seizures(126)
- **EDUCATE: Side effects:** Insomnia, tachycardia
- **EDUCATE: Considerations:** Avoid bupropion in patients with seizure disorders.(67)
- **SEND:** How to stop smoking  (Spanish)
- **Goal:** Nicotine replacement therapy (NRT) initiated and maintained(67)
 - **Interventions:**
 - **ASSIST:** Appointment scheduling with prescribing provider for medication evaluation and refills
 - **EDUCATE: Purpose:** NRT is used to reduce withdrawal feeling during smoking cessation.
 - Dosage is individualized based on need and response of patient.
 - If using patch, apply one new patch every 24 hours on skin that is dry, clean, and hairless. Rotate skin sites. Do not cut patch, and do not use more than one patch at a time.
 - If using lozenge or gum, do not eat or drink for 15 minutes before or while utilizing.
 - **EDUCATE: Call provider immediately:** Nicotine toxicity (eg, nausea, abdominal pain, vomiting, diarrhea, sweating, disturbed vision or hearing, palpitations, altered respirations)
 - **EDUCATE: Side effects:** Anxiety, sleep disorders, depression, myalgia
 - Patch may cause redness or swelling of the skin around the patch that does not go away.
 - **EDUCATE: Considerations:** Avoid nicotine use in immediate post-MI period or in presence of serious arrhythmias or severe or worsening angina.
 - **SEND:** How to stop smoking  (Spanish)

P19. Tobacco use in patient history

- **Goal:** Tobacco abstinence maintained(120)(127)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling with provider for evaluation of improved or stabilized health status, for reinforcement of abstinence from tobacco
 - **EDUCATE:** How to keep environment tobacco free
 - **EDUCATE:** Encourage continued abstinence from smoking and review tobacco cessation strategies.
 - **EDUCATE: Resources:**
 - American Lung Association: www.lung.org
 - Centers for Disease Control and Prevention: www.cdc.gov/tobacco
 - Smokefree. National Institutes of Health: www.smokefree.gov
 - **COORDINATE:** Tobacco use treatment, as needed
 - **COORDINATE:** Support group referral, as needed
 - **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
 - **COORDINATE:** Follow-up contact to evaluate tobacco use, referral status, and need for further assistance(19)(20)

P20. Potential hazardous or at-risk alcohol use present

- **Goal:** Alcohol effects and complications of misuse understood(128)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling for evaluation of effects of alcohol use and self-care plan, if needed
 - **EDUCATE:** Health effects of drinking(128)(129):
 - Light to moderate intake (2 drinks per day for men and 1 drink per day for women) may be linked with poor health outcomes.[C](130)(131)
 - Heavy drinking can cause acute alcohol poisoning and nutritional deficiencies.
 - Long-term effects of alcohol misuse include cirrhosis, pancreatitis, peripheral neuropathy, and cancer.(120)
 - **EDUCATE:** Follow drinking limits set by provider.
 - **EDUCATE:** Comparison of personal drinking to national norms(129):
 - Safe drinking levels for men (younger than 65 years) are 14 drinks or fewer per week and 4 drinks or fewer on one occasion.

- Safe drinking levels for women are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
- Safe drinking levels for healthy adults age 65 years are 7 drinks or fewer per week and 3 drinks or fewer on one occasion.
- No drinking during pregnancy is safe. Caution should be used when breast-feeding; consumption of a single drink may be safe if the mother waits 4 hours before breast-feeding or expresses breast milk for feeding prior to consuming the alcoholic drink.
- **EDUCATE:** Harmful alcohol interactions with medications (eg, with analgesics, antibiotics, anticoagulants, antidepressants, antihistamines, anticonvulsants, medications for diabetes, beta-blockers, and sedative/hypnotic medications)
- **EDUCATE:** Role of self-help programs (eg, Alcoholics Anonymous) and cognitive behavioral therapy(132)
- **EDUCATE: When to get emergency help(133):**
 - Drug overdose or alcohol poisoning (eg, loss of consciousness, problems breathing, coma, low blood pressure or body temperature)
 - Delirium tremens (DT), usually starts 48 to 72 hours after last drink (eg, confusion, disorientation, tachycardia, severe tremors, hallucinations, and seizures)(134)
- **EDUCATE: When to call the provider for behavior changes:**
 - Substance misuse (eg, increased use, use interferes with ADL/IADL)
 - Medication problems (eg, side effects, access, administration)
 - Depression (eg, feeling sad, hopeless, overwhelmed)(26)(48)
- **EDUCATE: Resources:**
 - Al-Anon Family Groups: www.al-anon.org
 - Alcoholics Anonymous World Services: www.aa.org
 - National Institute on Alcohol Abuse and Alcoholism: www.niaaa.nih.gov
 - Substance Abuse and Mental Health Services Administration: www.samhsa.gov
- **COORDINATE:** Behavioral health services referral, if indicated
- **COORDINATE:** Community services referral, as needed(132)
- **COORDINATE:** Support group referral, as needed
- **COORDINATE:** Social worker referral, as needed(59)
- **COORDINATE: Substance-related disorders** assessment, if indicated(132)
- **COORDINATE:** Care team meeting, if needed(15)(16)
- **COORDINATE:** Communication between care team members and care plan reconciliation(15)(17)(18)
- **COORDINATE:** Follow-up contact to evaluate alcohol use status, referral status, and need for further assistance(19)(20)
- **SEND:** Tips for good mental health  (Spanish)
- **SEND:** What you need to know - Substance-related disorders  (Spanish)
- **SEND:** Be ready for your doctor visit - Substance-related disorders  (Spanish)
- **SEND:** Be ready with questions for your doctor - Substance-related disorders  (Spanish)

P21. Vaccinations not up to date, increasing infection risk

- **Goal:** Vaccinations up to date(135)(136)
 - **Interventions:**
 - **ASSIST:** Identifying patient's goal priorities, willingness/ability to achieve goals, and preferences for communication and follow-up
 - **ASSIST:** Goal setting that is specific, measurable, achievable, relevant, and with target completion time (eg, short/long-term, prioritized)
 - **ASSIST:** Appointment scheduling
 - **ASSIST:** Identifying vaccines patient may need that are not contraindicated based on clinical status
 - **EDUCATE:** Purpose and benefits of vaccinations
 - **EDUCATE:** Recommended immunizations and timing of administration:
 - Centers for Disease Control and Prevention's vaccination schedule: www.cdc.gov/vaccines(137)
 - Vaccine schedules may need to be altered based on medications; check with the specialist provider for information.(138)
 - Possible side effects or complications associated with getting vaccinated
 - **EDUCATE:** Influenza vaccination annually(97):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects with shot are sore, red, or swollen area where shot was given; fever; and aches.
 - Common side effects with nasal spray are mild and last 1 or 2 days; they include runny nose, headache, sore throat, and cough.
 - Allergic reaction rare and is usually caused by a chicken egg allergy
 - **EDUCATE:** Pneumococcal vaccination potential side effects:
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; fever; and muscle aches.

- Serious side effects are rare.
- **EDUCATE:** COVID-19 vaccination potential side effects(137):
 - Side effects are mild and last 1 or 2 days.
 - Common side effects include a sore, red, or swollen area where shot was given; chills; fatigue; and aches.
 - Serious side effects are rare.
- **COORDINATE:** Community services referral, as needed
- **COORDINATE:** Vaccinations
- **COORDINATE:** Follow-up to evaluate vaccination understanding, referral status, and need for further assistance(19)(20)
- **SEND:** Coronavirus disease 2019 (COVID-19) vaccines  (Spanish)
- **SEND:** Flu vaccine  (Spanish)
- **SEND:** Pneumonia vaccine for adults  (Spanish)
- **SEND:** Hepatitis A vaccine  (Spanish)
- **SEND:** Hepatitis B vaccine  (Spanish)

Evidence Summary

Quality standards: The Case Management Society of America's Standards of Practice for Case Management principles include facilitation of informed decision making, counseling, and health education with patient self-care that is responsive to the patient's preferences (eg, culture, values, needs).(139)

Outcomes: An observational study based on 554 persons with dementia and their caregivers demonstrated the clinical benefit of a comprehensive dementia care management program for most participants at 1 year. The dementia care manager (nurse practitioner) partnered with primary care physicians, specialty physicians, and community-based organizations to provide education and support, which showed improvement with behavioral symptoms in persons with dementia as well as a decrease in caregiver distress.(140)

Quality standards: Standards indicate that program content should include an individualized self-care plan that includes patient preferences, prioritized goals, interventions with information on whom to contact in urgent situations, and scheduled follow-up. Programs should facilitate communication and collaboration with practitioners in a timely manner.(139)(141)(142)(143)

National guidelines: The 2018 Physical Activity Guidelines Advisory Committee advises that adults with chronic conditions or disabilities, who are able, should follow the key activity guidelines for adults. The guidelines recommend that adults should do at least 150 to 300 minutes a week of moderate-intensity or 75 to 150 minutes a week of vigorous-intensity aerobic physical activity, or an equivalent combination of moderate-intensity and vigorous-intensity aerobic activity. They should also do muscle-strengthening activities on 2 or more days a week. Older adults should do multicomponent physical activity that includes balance training as well as aerobic and muscle-strengthening activities.(144)

Evidence: Cognitive stimulation therapy, which encourages information processing in a pleasurable context and can be administered in either a group or individual format, has been shown in studies to improve quality of life, modestly improve cognitive functioning, and slow cognitive decline in patients with mild to moderate dementia above and beyond any benefits that may be achieved with medication therapy.(38)

Evidence: A systematic review of 33 randomized controlled trials found that cognitive therapy in patients with mild to moderate dementia may be associated with positive effects on global cognition and verbal semantic fluency at the end of treatment.(145)

Evidence: A systematic review of 37 studies including 2766 participants with mild or moderate dementia found that participants in cognitive stimulation programs improved on measures of communication and social interactions and showed slight benefits in day-to-day activities and in their own perceptions of their mood.(146)

Specialty society: The National Institute for Health and Care Excellence (NICE) advises against the use of ginseng, vitamin E supplements, or herbal formulations to treat dementia.(38)

Specialty society: The American Geriatrics Society does not recommend feeding tubes in older adults with advanced dementia. The society states that tube feedings are associated with agitation, greater use of physical and chemical restraints, greater healthcare use related to tube complications, and development of new pressure injuries. Careful hand feeding should be offered, as outcomes of death, aspiration pneumonia, functional status, and comfort are as good as with tube feedings.(147)

Quality standards: Standards indicate that programs should ensure that during care transitions the care plan is re-evaluated, modified, and communicated to appropriate care team members (eg, providers, caregivers) as needed to support the care transition.(139)(141)(142)(143)

Evidence: Comorbid psychiatric disorders (eg, depression, psychosis, and anxiety), as well as acute behavioral disturbances (eg, agitation), are common in patients with dementia and have a significant impact on patient quality of life, caregiver stress, and the likelihood of institutionalization.(9)(46)

Transition consideration: Several studies demonstrated a reduction in 30-day hospital readmissions when the patient follow-up was within 7 days of hospital discharge.(148)(149)(150) Earlier follow-up (eg, within 48 hours after hospital discharge) may also help reduce hospital readmissions for high-risk or vulnerable patients.(19) Follow-up appointment attendance rates were higher when appointments were scheduled before hospital discharge.(151)(152)

Outcomes: Several studies support the role of the pharmacist in medication reconciliation to identify and resolve pharmacy-related problems and improve outcomes (eg, prevention of medication-related adverse events, discontinuation of inappropriate medication).(153)

Evidence: A systematic review and meta-analysis including 44 randomized controlled trials and 9811 patients with dementia found a small clinical benefit for memantine (with or without concomitant cholinesterase inhibitors) in moderate to severe Alzheimer disease in clinical global rating, cognitive function, and performance in activities of daily living.(154) A review of the effects of memantine on clinical progression in Alzheimer disease concluded that memantine may slow the decline in function, and in some cases symptoms may improve.(64)

National guidelines: The National Institute for Health and Care Excellence (NICE) recommends that patients with dementia try antipsychotics only if severely distressed or at risk of harming themselves or others, and they try them along with other interventions to relieve distress.(69)

Evidence: Current evidence suggests that antipsychotics are best reserved as a last resort for severe refractory aggression, agitation, or psychosis, or when a serious threat of immediate harm or safety risk exists.(12)

National guidelines: The U.S. Preventive Services Task Force recommends exercise interventions (eg, gait training, balance training) to help prevent falls in at-risk adults age 65 years or older.(81)

National guidelines: The U.S. Preventive Services Task Force recommends exercise-based interventions to help reduce falls for people 65 years and older. Other interventions (eg, behavioral therapy, nutrition therapy, environmental modification, medication changes) may be considered based on the patient's preferences, the benefit-to-harm balance, and the circumstances of prior falls. Vitamin D supplementation is not recommended for fall prevention for older people without a known deficiency or osteoporosis.(81)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, sleep patterns). Programs should have a process for following up with patients on the status of referrals.(139)

Specialty society: In a systematic review of the pharmacologic treatment of chronic insomnia in adults, the American Academy of Sleep Medicine does not recommend diphenhydramine, melatonin, tryptophan, or valerian for treating sleep onset or maintenance.(100)

Specialty society: The American College of Physicians recommends cognitive behavioral therapy for insomnia as the first-line treatment for patients with chronic insomnia (trouble falling or staying asleep for 3 or more nights a week for at least 3 months, with daytime consequences). Cognitive behavioral therapy for insomnia generally includes cognitive therapy with sleep restriction, relaxation training, and stimulus control for 5 to 8 sessions.(155)

National guidelines: The Institute of Medicine does not specify how much total fluid should come from water. Their recommendation is based on the median of total water intake on dietary surveys, which include water from liquids and solids. For healthy people, the advice is to consume at least 8 glasses (8 ounces (240 mL) each) of water daily; there is little evidence to support this recommendation.(156)

Evidence: The Alcohol Use Disorders Identification Test (AUDIT) questionnaire's 3-item subset of questions about alcohol consumption (AUDIT-C) is a brief screening test for problem drinking that has been validated in primary care office settings.(129)

Quality standards: Standards indicate that program content should include a condition treatment plan, self-management and self-monitoring, and health behaviors (eg, substance use). Programs should have a process for following up with patients on the status of referrals.(139)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) recommends that clinicians ask all adults about tobacco use, advise them to stop using tobacco, and provide behavioral interventions and FDA-approved pharmacotherapy for cessation to nonpregnant adults who use tobacco. The USPSTF recommends that clinicians ask all pregnant persons about tobacco use, advise them to stop using tobacco, and provide behavioral interventions for cessation to those who use tobacco.(157)

National guidelines: The U.S. Preventive Services Task Force (USPSTF) concludes that there is insufficient evidence to determine the balance of benefits and harms of primary care interventions for tobacco cessation among school-age children and adolescents who already smoke because of a lack of adequately powered studies on behavioral counseling interventions and a lack of studies on medications. Clinical judgment should be used to decide how to best help youth who use tobacco.(158)

Evidence: A systematic review of 33 trials with 21,600 participants found that incentives (eg, money, vouchers) improved smoking cessation rates at long-term follow-up (at least 6 months) in mixed-population studies. Ten trials of 2571 pregnant women found that women in the incentive-rewards group were more likely to stop smoking.(159)

Evidence: A systematic review of 47 studies with more than 18,000 people showed improved smoking cessation with increased therapeutic support and nicotine replacement therapy (NRT) as compared with NRT alone.(122)

Evidence: A review of 43 studies with more than 28,000 people found with moderate certainty that when no other support is available, written self-help materials help more people to stop smoking compared with getting no help at all.(160)

Evidence: A meta-analysis of 10 randomized studies of 3655 participants using nicotine replacement therapy found that behavioral support focusing on adherence to smoking cessation medications can modestly improve adherence.(161)

Evidence: A network meta-analysis of 33 systematic reviews with 250,563 participants found that behavioral support for smoking cessation can increase quit rates at 6 months or longer, with the strongest benefit for the provision of any type of counseling and guaranteed financial incentives.(162)

Evidence: A systematic review of 136 trials of nicotine replacement therapy (NRT) with 64,640 people found that all forms of NRT made it more likely that a person's attempt to quit smoking would succeed. Their chances of stopping smoking increased by 50% to 60% with or without additional counseling. There is no evidence that NRT increases the risk of heart attacks. In pregnant women, evidence suggests that NRT can increase the chance of quitting at the time of delivery.(163)

Evidence: A systematic review of 24 trials with 7279 participants found no evidence that adding exercise to smoking cessation support improves abstinence compared with support alone, but the evidence is insufficient to assess whether there is a modest benefit.(164)

Evidence: A systematic review of 51 studies including 22,000 people who smoked tobacco found that cessation was more likely if smokers cut down their intake when using stop-smoking medication (eg, varenicline) or fast-acting nicotine replacement therapy vs quitting all at once.(119)

Specialty Society: The American Thoracic Society recommendations for tobacco-dependent adults include using varenicline rather than a nicotine patch, using varenicline rather than bupropion, using varenicline rather than a nicotine patch in adults with a comorbid psychiatric condition, initiating varenicline in adults even if they are unready to quit, and using controller therapy for an extended treatment duration greater than 12 weeks.(125)

National guidelines: The US Department of Health and Human Services' Dietary Guidelines for Americans suggests that moderate alcohol consumption (2 drinks/day in men or 1 drink/day in women) may increase the overall risk of death from various causes, such as several types of cancer and some forms of cardiovascular disease.(110)

Specialty society: The American Heart Association does not support initiation of alcohol intake to improve cardiovascular health.(130)

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Footnotes

[A] Assess and address the emergency or urgent problems before continuing the engagement. [A in Context Link 1]

[B] A self-care plan tells you how to take care of your condition and when and where to get help for problems. [B in Context Link 1]

[C] A standard drink consists of 0.6 ounces (18 mL) of ethanol, which is contained in 12 ounces (360 mL) of beer (alcohol by volume (ABV) = 5%), 5 ounces (150 mL) of wine (ABV = 12%), and 1.5 ounces (45 mL) of spirits (ABV = 40%).(110) [C in Context Link 1, 2]

[D] Genetic screening is available for mutations associated with certain forms of dementia; however, testing should be done with formal genetic counseling, and positive results do not guarantee the patient will develop the disease.(7) [D in Context Link 1]

[E] Exercise and therapies (eg, physical, occupational, and speech therapy) have a sustainable effect in terms of maintaining symptoms and improving quality of life, but will have a reduction in benefit as the disease progresses.(165) [E in Context Link 1]

[F] Vascular dementia is associated with hypertension, transient ischemic attacks (TIAs), and cardiovascular disease and has the same preventive measures and risk factors (eg, hypercholesterolemia, heart disease, diabetes).(27)(40) [F in Context Link 1]

[G] The Mediterranean diet, which recommends fish intake, is associated with decreased risk of Alzheimer disease (AD) and decreased risk of converting from mild cognitive impairment (MCI) to AD dementia.(166) [G in Context Link 1]

[H] The timing of provider follow-up depends on the patient's disease severity, self-care complexity, patient/caregiver willingness and ability to provide patient care, and available social supports.(167) [H in Context Link 1]

[I] Rule out all obvious causes of agitation (eg, pain) and provide behavioral interventions prior to administration of typical antipsychotic medication.(69)(70) [I in Context Link 1]

[J] Neuroleptic malignant syndrome is a rare, rapidly progressive, potentially fatal complication more common with typical antipsychotics. Onset can occur hours or years after initiation of drug therapy.(71) [J in Context Link 1]

[K] Tardive dyskinesia (TD) is more common with first-generation (typical) antipsychotics. The risk of TD is higher with long-term use of antipsychotics. The offending medication should be removed with the first sign of symptoms, as symptoms may be irreversible.(168) [K in Context Link 1]

[L] Use typical antipsychotic medication when psychotic symptoms outweigh the adverse effects of medication.(72) [L in Context Link 1]

[M] A pharmacist can support medication reconciliation for redundancy or duplication and identify drug interactions (eg, with other drugs, food).(73) [M in Context Link 1]

[N] Services to consider for community support include stairway modifications, widening of interior doors for ambulation equipment, and installation of handrails.(106) [N in Context Link 1]

[O] Many driver rehabilitation programs provide therapy and behind-the-wheel assessments. Therapy assessment covers motor, cognitive, perception, and reaction times. The behind-the-wheel assessment covers fitness to drive.(169) [O in Context Link 1]

[P] Adequate intake is used when the recommended daily allowance cannot be determined because of insufficient data on requirements.(111) [P in Context Link 1]

[Q] Primary pharmacotherapy for tobacco dependence includes varenicline, bupropion, nicotine gum, nicotine inhaler, nicotine nasal spray, and nicotine patch. Nicotine replacement therapies (NRTs) should be used with caution for patients in the 2 weeks following myocardial infarction and for those with life-threatening arrhythmias or severe angina.(116)(157) [Q in Context Link 1]

Codes

ICD-10 Diagnosis: F01.50, F01.511, F01.518, F01.52, F01.53, F01.54, F01.A0, F01.A11, F01.A18, F01.A2, F01.A3, F01.A4, F01.B0, F01.B11, F01.B18, F01.B2, F01.B3, F01.B4, F01.C0, F01.C11, F01.C18, F01.C2, F01.C3, F01.C4, F02.80, F02.811, F02.818, F02.82, F02.83, F02.84, F02.A0, F02.A11, F02.A18, F02.A2, F02.A3, F02.A4, F02.B0, F02.B11, F02.B18, F02.B2, F02.B3, F02.B4, F02.C0, F02.C11, F02.C18, F02.C2, F02.C3, F02.C4, F03.90, F03.911, F03.918, F03.92, F03.93, F03.94, F03.A0, F03.A11, F03.A18, F03.A2, F03.A3, F03.A4, F03.B0, F03.B11, F03.B18, F03.B2, F03.B3, F03.B4, F03.C0, F03.C11, F03.C18, F03.C2, F03.C3, F03.C4, F06.70, F06.71, F10.27, F10.97, F13.27, F13.97, F18.17, F18.27, F18.97, F19.17, F19.27, F19.97, G30.0, G30.1, G30.8, G30.9, G31.01, G31.09, G31.1, G31.83, G31.85, G93.44 [Hide]

DSM-5: F01.50, F01.511, F01.518, F01.52, F01.53, F01.54, F01.A0, F01.A11, F01.A18, F01.A2, F01.A3, F01.A4, F01.B0, F01.B11, F01.B18, F01.B2, F01.B3, F01.B4, F01.C0, F01.C11, F01.C18, F01.C2, F01.C3, F01.C4, F02.80, F02.811, F02.818, F02.82, F02.83, F02.84, F02.A0, F02.A11, F02.A18, F02.A2, F02.A3, F02.A4, F02.B0, F02.B11, F02.B18, F02.B2, F02.B3, F02.B4, F02.C0, F02.C11, F02.C18, F02.C2, F02.C3, F02.C4, F03.90, F03.911, F03.918, F03.92, F03.93, F03.94, F03.A0, F03.A11, F03.A18, F03.A2, F03.A3, F03.A4, F03.B0, F03.B11, F03.B18, F03.B2, F03.B3, F03.B4, F03.C0, F03.C11, F03.C18, F03.C2, F03.C3, F03.C4, F06.70, F06.71, F10.27, F10.97, F13.27, F13.97, F18.17, F18.27, F18.97, F19.17, F19.27, F19.97 [Hide]

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